

TOMO RIBA INSTITUTE OF HEALTH & MEDICAL SCIENCES (TRIHMS)

Department of Neurology

Old Assembly Complex, Naharlagun, Itanagar, Arunachal Pradesh 791110

CONFIDENTIAL CLINICAL MEDICAL RECORD - NEUROLOGY CONSULTATION

Patient Name: Radhe, Tage	MRN: TRIHMS-2026-5531	Sex: Male
DOB / Age: 14/07/1960 (Age: 66)	Date of Exam: 26/08/2026	Primary Caregiver: Tage Yapi (Wife / Primary Caregiver)
Attending Neurologist: Dr. Bengia Tado, MD, DM (Neurology)	Referring Physician: Dr. Koj Tasso, MD (Medicine)	Clinical Status: Outpatient Evaluation

REASON FOR EVALUATION

Evaluation of progressive memory deficits, word-finding hesitations, and executive difficulty handling complex agricultural logistics over 18 months.

CLINICAL HISTORY (HISTORY OF PRESENT ILLNESS)

The patient is a 66-year-old right-handed Apatani male, retired state horticulture officer from Ziro valley currently residing in Naharlagun, presenting with 18 months of memory decline. His wife noted that he repeatedly forgets recent conversations, misplaces land records, and struggles with calculations for agricultural fertilizer purchases. He was recently unable to find his parked car outside the Naharlagun daily market. Functional Assessment: Requires assistance for complex IADLs (financial budgeting, tax filing, medication organization). Independent in meal preparation, simple domestic chores, and all basic ADLs. Ceased driving long highway routes due to navigation anxiety. Behavioral Symptoms: Mild apathy, reduced social visits to community council meetings; no hallucinations, delusions, or agitation. Sleep pattern normal.

PAST MEDICAL HISTORY	ACTIVE MEDICATIONS
- Pre-diabetes (HbA1c 6.2%) - Mild Hypertriglyceridemia - No prior neurological trauma or family history of early dementia	- Atorvastatin 10 mg PO daily - Vitamin B-Complex daily

PHYSICAL & NEUROLOGICAL EXAMINATION

Alert, cooperative, well-groomed, pleasant. Cranial Nerves: I-XII intact. Motor: Power 5/5 in all 4 extremities, normal muscle bulk and tone, no tremor or rigidity. Reflexes: 2+ symmetric, plantars flexor. Gait & Balance: Normal stride length, tandem gait intact, Romberg negative.

STANDARDIZED COGNITIVE TESTING

Mini-Mental State Examination (MMSE): Score 22/30 (Mild Cognitive Impairment / Early Dementia) | MoCA: Score 20/30

- Orientation: 7/10 (Disoriented to day of week, exact date, season)
- Registration: 3/3
- Attention & Calculation: 3/5 (Errors on Serial 7s)
- Recall: 1/3 (Delayed recall defective; improves mildly with semantic cues)
- Language & Visuospatial: 8/9 (Clock Drawing shows minor symmetry error; mild anomia on confrontation naming)

DIAGNOSTIC WORKUP & NEUROIMAGING BIOMARKERS

- Laboratory: Normal CBC, CMP, TSH (1.8 mIU/L), B12 (550 pg/mL), Folate (>20 ng/mL). Syphilis and HIV serologies non-reactive.
- Brain MRI: Symmetrical bilateral moderate hippocampal atrophy (MTA Score: Grade 2). Mild bilateral parietal cortical thinning. No microvascular ischemic burden (Fazekas Grade 0). No focal lesions.

DIAGNOSTIC IMPRESSION

- Major Neurocognitive Disorder due to Early-Onset Alzheimer's Disease (ICD-10: G30.0 / F02.80), Mild Stage.
- Classical amnesic-predominant presentation with supportive medial temporal atrophy on volumetric neuroimaging.

TREATMENT PLAN & CLINICAL RECOMMENDATIONS

- Pharmacotherapy: Initiate Donepezil 5 mg PO at bedtime for 4 weeks, with planned escalation to 10 mg daily depending on tolerance.
- Cognitive & Physical Rehabilitation: Enrollment in structured memory wellness exercises; daily outdoor walking.
- Safety Planning: Transition vehicle driving privileges to family members; establish automated pill organizer and electronic bill payments.
- Follow-Up: Clinical and cognitive re-evaluation scheduled in 3 months.

Electronically Verified & Signed by:

Dr. Bengia Tado, MD, DM (Neurology)

Associate Professor of Neurology | Reg No: APMC-03819

TRIHMS, Naharlagun, Arunachal Pradesh

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